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MC-2202

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Passport No :

LABORATORY TEST REPORT

Patient Information	Sample Information	Client/Location Information
Name : Lyubochka Svetka	Lab Id : 02232160XXXX	Client Name : Sterling Accuris Buddy
Sex/Age : Male / 41 Y 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:38 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Thyroid Function Test

Test	Result	Unit	Biological Ref. Interval
T3 - Triiodothyronine <i>Chemiluminescence</i>	1.01	ng/mL	0.58 - 1.59
T4 - Thyroxine	7.84	mg/mL	4.87 - 11.72
TSH - Thyroid Stimulating Hormone <i>Chemiluminescence</i>	0.8199	microIU/mL	0.35 - 4.94

TSH	T3/FT3	T4/FT4	Suggested Interpretation for the Thyroid Function Tests Pattern
Within Range	Decreased	Within Range	- Isolated Low T3-often seen in elderly & associated Non-Thyroidal illness. In elderly the drop in T3 level can be upto 25%.
Raised	Within Range	Within Range	- Isolated High TSH especially in the range of 4.7 to 15 mIU/ml is commonly associated with physiological & Biological TSH Variability. - Subclinical Autoimmune Hypothyroidism - Intermittent T4 therapy for hypothyroidism - Recovery phase after Non-Thyroidal illness
Raised	Decreased	Decreased	- Chronic autoimmune Thyroiditis - Post thyroidectomy, Post radioiodine - Hypothyroid phase of transient thyroiditis
Raised or Within Range	Raised	Raised or Within range	- Interfering antibodies to thyroid hormones (anti-TPO antibodies) - Intermittent T4 therapy or T4 overdose - Drug interference-Amiodarone, Heparin, Beta blockers, steroids, anti-epileptics
Decreased	Raised or within Range	Raised or within Range	- Isolated Low TSH - especially in the range of 0.1 to 0.4 often seen in elderly & associated with Non-Thyroidal illness - Subclinical Hyperthyroidism - Thyroxine ingestion
Decreased	Decreased	Decreased	- Central Hypothyroidism - Non-Thyroidal illness - Recent treatment for Hyperthyroidism (TSH remains suppressed)
Decreased	Raised	Raised	- Primary Hyperthyroidism (Graves disease), Multinodular goitre Toxic nodule - Transient thyroiditis:Postpartum, Silent (lymphocytic), Postviral (granulomatous, subacute, DeQuervain's) Gestational thyrotoxicosis with hyperemesis gravidarum
Decreased or within range	Raised	Within Range	- T3 toxicosis - Non-Thyroidal illness

P.D.

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MD(Path)

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Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:40 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Urine	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test	Result	Unit	Biological Ref. Interval
Microalbumin (per urine volume) <i>Immunoturbidimetric</i>	10.50	mg/L	< 16.7

In random urine specimens, normal urinary albumin excretion is below 17 mg/g creatinine for males and below 25 mg/g creatinine for females.(3)

Microalbuminuria is defined as an albumin:creatinine ratio of 17 to 299 for males and 25 to 299 for females.

A ratio of albumin:creatinine of 300 or higher is indicative of overt proteinuria.

Due to biologic variability, positive results should be confirmed by a second, first-morning random or 24-hour timed urine specimen. If there is discrepancy, a third specimen is recommended. When 2 out of 3 results are in the microalbuminuria range, this is evidence for incipient nephropathy and warrants increased efforts at glucose control, blood pressure control, and institution of therapy with an angiotensin-converting-enzyme (ACE) inhibitor (if the patient can tolerate it).

Reference :

1. Bennett PH, Haffner S, Kasiske BL, et al: Screening and management of microalbuminuria in patients with diabetes mellitus: recommendations to the Scientific Advisory Board of the National Kidney Foundation from an ad hoc committee of the Council on Diabetes Mellitus of the National Kidney Foundation. Am J Kidney Dis 1995;25:107-112
2. Krolewski AS, Laffel LM, Krolewski M, et al: Glycosylated hemoglobin and the risk of microalbuminuria in patients with insulin-dependent diabetes mellitus. N Engl J Med 1995;332:1251-1255
3. Zelmanovitz T, Gross JL, Oliveira JR, et al: The receiver operating characteristics curve in the evaluation of a random urine specimen as a screening test for diabetic nephropathy. Diabetes Care 1997;20:516-519

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	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Protein

Test	Result	Unit	Biological Ref. Interval
Total Protein <i>Copper tartrate to colour complex</i>	7.00	g/dL	6.3 - 8.2
Albumin <i>Bromocresol Green Method</i>	4.20	g/dL	3.5 - 5.0
Globulin <i>Calculated</i>	2.80	g/dL	2.3 - 3.5
A/G Ratio <i>Calculated</i>	1.50		1.3 - 1.7

Bilirubin

Total Bilirubin <i>Azobilirubin chromophores</i>	0.70	mg/dL	0.2 - 1.3
Conjugated Bilirubin <i>Cationic Mordant Binding</i>	0.30	mg/dL	0.0 - 0.3
Unconjugated Bilirubin <i>Cationic Mordant Binding</i>	0.20	mg/dL	0.0 - 1.1
Delta Bilirubin <i>Calculated</i>	0.20	mg/dL	0.0 - 0.2

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Iron Studies

Test	Result	Unit	Biological Ref. Interval
Iron <i>Pyridyl azo Dye</i>	103.00	micro g/dL	49 - 181
Total Iron Binding Capacity (TIBC)	352.00		261 - 462
Transferrin Saturation <i>Calculated</i>	29.26	%	Children : >16 Adult : 20 - 50

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Immunoassay

Test	Result	Unit	Biological Ref. Interval
Homocysteine, Serum Chemiluminescence	H 23.86	micromol/L	6.0 - 14.8

Summary and Uses:

- Total Hcy is a thiol-containing amino acid, produced by the intracellular demethylation of methionine to cysteine.
- Elevated levels of t Hcy may be used to exclude or confirm deficiencies of vitamin B12 or folate.
- It is recommended to test in patients using medications that interfere with folate status (methotrexate, antiepileptics), vegetarians without B12 supplementations, unexplained anemia, peripheral neuropathy or myelopathy, recurrent spontaneous abortions or infertility.
- Testing also recommended for patients 40 years of age with coronary artery disease to exclude homocystinuria.
- Elevations in tHcy levels have also been used as an independent risk factor of coronary or cerebral vascular disease. Treatment of moderate hyperhomocystinemia with folic acid supplementation for primary and secondary cardiovascular protection has met with inconsistent results and at present cannot be routinely recommended.

Limitations:

- The plasma must be separated immediately on collection to avoid continuous synthesis of Hcy by red cells.
- Samples must be immediately stored on ice and serum centrifuged immediately before a complete clot is formed.
- Certain drugs, such as anticonvulsants, methotrexate, or nitrous oxide, may interfere with the assay.
- Cigarette smoking and coffee consumption increase tHcy levels.
- Intraindividual variability is approximately 8%; it can be as much as 25% in patients with hyperhomocystinemia.
- Generally, a single measurement of tHcy is considered adequate.

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